

Department of Obstetrics And Gynaecology
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 31.1 Yr/F
Admission No.: anonymous	Department Unit: Obstetrics And Gynaecology (Obs Unit 1)	Consultant/Faculty: anonymous
Ward: 1 B 1 Female Ward	Room/Bed: 1 B 1 Female Room/ 1 B 1 Bed 13	Patient Category: Ayushman
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation: House Wife
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 31-Mar-2023 13:07	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:16

PROVISIONAL DIAGNOSIS:

30 YR/G3P2L2/37 4 WK POG/VD F/B LSCS 3.5YR BACK/RH -VE PREG/ANTI D TAKEN AT 29 WK
POG/HYPOTHYROIDISM/POLYHYDRAMNIOS(AFI31.8cm) ADMITTED FOR SAFE CONFINEMENT PERCEIVING FETAL MOVEMENTS WELL.
30 YR/G3L3/POD 3 OF EL-LSCS TVO PREVIOUS CS NOT WILLING FOR TOLAC

OPERATION PERFORMED:

ELECTIVE LSCS I/V/O PREVIOUS CS NOT WILLING FOR TOLAC B/L TUBAL LIGATION On 01-Apr-2023

SURGEONS: Dr Jyotsna

ASSISTANT SURGEONS/SENIOR RESIDENTS: DR POLARICA

OPERATIVE FINDINGS: UNDER ADEQUATE SPINAL ANAESTHESIA, PFANNENSTIAL INCISION GIVEN. ABDOMEN OPENED IN LAYERS LUS WAS IDENTIFIED, A SMALL NICK WAS GIVEN AT LUS AND EXTENDED BILATERALLY.AMNIOTIC FLUID DRAINED OF 1500ml- CLEAR. TERM/ALIVE /MALE BABY OF 2.895 KG DELIVERED BY VERTEX PRESENTATION ON 1/4/23 AT 10:41 AM . BABY CRIED IMMEDIATELY AFTER BIRTH. PLACENTA AND MEMBRANES DELIVERED COMPLETELY.B/L TUBAL LIGATION WAS DONE WITH MODIFIED POMEROYS METHOD WITH CATGUT.ADEQUATE HEMOSTASIS ACHIEVED. INTRAOP BLOOD LOSS- 500ML, URINE OUTPUT- 100ML. BABY WAS BREASTFED WITHIN 1 HOUR AFTER DELIVERY

CHIEF COMPLAINTS:

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POG/HYPOTHYROIDISM/POLYHYDRAMNIOS(AFI31.8cm) ADMITTED FOR SAFE CONFINEMENT PERCEIVING FETAL MOVEMENTS WELL

HISTORY OF PRESENT ILLNESS: 30 YR/G3P2L2/37 4 WK POG/VD F/B LSCS 3.5YR BACK/RH -VE PREG/ANTI D TAKEN AT 29 WK
POG/HYPOTHYROIDISM/POLYHYDRAMNIOS(AFI31.8cm) ADMITTED FOR SAFE CONFINEMENT PERCEIVING FETAL MOVEMENTS WELL.ON CLINICAL EXAMINATION, SHE WAS VITALLY STABLE, PERCEIVEING FETAL MOVEMENTS WELL, HER NST WAS REACTIVE AT THE TIME OF ADMISSION.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
02-Apr-2023 00:06	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.04	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.02	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	2.07	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	1.01	0.2 - 1.0 x10 ³ /μL

31-Mar-2023 18:02	ACTIVATED PARTIAL THROMBOPLASTIN TIME (APTT)	Absolute Neutrophil Count	16.74	2 - 7 x10 ³ /μL
		Basophil	0.2	00 - 01 %
		Eosinophils	0.1	01 - 06 %
		HCT	35.5	36 - 46 %
		Hemoglobin	11.2	12 - 15 g/dl
		Lymphocytes	10.4	20 - 40 %
		MCH	30.1	27 - 32 pg
		MCHC	31.5	31.5 - 34.5 g/dL
		MCV	95.4	83 - 101 fL
		Monocytes	5.1	2 - 10 %
		Neutrophils	84.2	40 - 80 %
		Platelet Count	263	150 - 410 x10 ³ /μL
		RBC	3.72	3.8 - 4.8 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	13.5	11.6 - 14.0 %
		Total Leukocyte Count(TLC)	19.88	4 - 10 x10 ³ /μL
		APTT HEADER	25.1	23.2 - 32.4 sec
		APTT NOTE	Refer Reports*	-
		clinical details	--	-
		Comments on PBS	Refer Reports*	-
		COMPLETE BLOOD COUNT (CBC)		
		Absolute Basophil Count	0.05	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.19	0.02 - 0.5 x10 ³ /μL
		Absolute Lymphocyte Count	3.22	1 - 3 x10 ³ /μL
		Absolute Monocyte Count	0.69	0.2 - 1.0 x10 ³ /μL
		Absolute Neutrophil Count	9.69	2 - 7 x10 ³ /μL
		Basophil	0.4	00 - 01 %
		Eosinophils	1.4	01 - 06 %
		HCT	37.3	36 - 46 %
		Hemoglobin	11.9	12 - 15 g/dl
		Lymphocytes	23.3	20 - 40 %
		MCH	30.3	27 - 32 pg
		MCHC	31.9	31.5 - 34.5 g/dL
		MCV	94.9	83 - 101 fL
		Monocytes	5.0	2 - 10 %
		Neutrophils	69.9	40 - 80 %
		Platelet Count	241	150 - 410 x10 ³ /μL
		RBC	3.93	3.8 - 4.8 x10 ⁶ /μL



Red Cell Distribution Width (RDW CV)	13.6	11.6 - 14.0 %
Total Leukocyte Count(TLC)	13.84	4 - 10 x10 ³ /μL
PROTHROMBIN TIME clinical details (PT INR)	--	-
International Normalised Ratio (INR)	1.1	-
PT INR Header	10.2	09.6 - 12.2 sec
PT INR Note	Refer Reports*	-

***Refer Lab Reports for details.**

OTHER INVESTIGATIONS: BLOOD GRP- O NEGATIVE

OBSTETRIC SONOGRAPHY DOPPLER (15/3/23):

LIVE INTRAUTERINE GESTATION WITH GESTATIONAL AGE 34 WEEKS 4 DAYS. EFW-2484 GM/ PLACENTA ANTERIOR GRADE 3/CEPHALIC PRESENTATION, NORMAL FETAL GROWTH AS PER GESTATION. LIQUOR ADEQUATE/ SINGLE LOOP OF CORD IS SEEN NEAR NECK AT TIME OF EXAMINATION/ NORMAL DOPPLER STUDY

HOSPITAL COURSE: 30 YR/G3P2L2/37 4 WK POG/VD F/B LSCS 3.5YR BACK/RH -VE PREG/ANTI D TAKEN AT 29 WK

POG/HYPOTHYROIDISM/POLYHYDRAMNIOS(AFI31.8cm) ADMITTED FOR SAFE CONFINEMENT PERCEIVING FETAL MOVEMENTS WELL.ON CLINICAL EXAMINATION, SHE WAS VITALLY STABLE, PERCEIVEING FETAL MOVEMENTS WELL, HER NST WAS REACTIVE AT THE TIME OF ADMISSION.

CONDITION AT ADMISSION-

GENERAL EXAMINATION:

PR: 78/MIN, BP: 110/70 MMHG, RR: 14/MIN, TEMP: AFEBRILE

PALLOR/ ICTERUS/CYANOSIS/CLUBBING/ EDEMA- ABSENT

SYSTEMIC EXAMINATION: CVS: S1 S2 , RS- B/L AIR ENTRY

P/A- UTERUS 36 WEEK SIZE,CEPHALIC,RELAXED,SFH-35cm,FHR-142bpm

P/S- OS OPEN. FRANK LEAK PRESENT

P/V- CERVIX SOFT POSTERIOR IFC,EARLY EFFACED HEAD FLOATING

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POG/HYPOTHYROIDISM/POLYHYDRAMNIOS(AFI31.8cm) ADMITTED FOR SAFE CONFINEMENT ON CLINICAL EXAMINATION SHE WAS NOT IN LABOR AND WAS VITALLY STABLE AND HER NST WAS REACTIVE AT THE TIME OF ADMISSION . STRICT FETOMATERNAL SURVEILLANCE WAS DONE. ALL THE NECESSARY INVESTIGATIONS WERE DONE. . SHE WAS PLANNED FOR ELECTIVE LSCS I/V/O OF PREVIOUS CS NOT WILLING FOR TOLAC. WHERE SHE DELIVERED A TERM /ALIVE /MALE BABY OF 2.895 KG BY VERTEX PRESENTATION ON 1/4/23 AT 10:41 AM . BABY CRIED IMMEDIATELY AFTER BIRTH. NO INTRAOP AND POSTOP COMPLICATIONS NOTED. MOTHER AND BABY PASSED URINE AND STOOL. ON POD3 CHECK DRESSING WAS DONE AND SUTURE SITE WAS HEALTHY. CONDITION OF MOTHER AND BABY WAS FAIR AT THE TIME OF DISCHARGE

CONDITION AT DISCHARGE: GENERAL EXAMINATION:

PR: 78/MIN, BP: 110/70 MMHG, RR: 14/MIN, TEMP: AFEBRILE

PALLOR/ ICTERUS/CYANOSIS/CLUBBING/ EDEMA- ABSENT

SYSTEMIC EXAMINATION: CVS: S1 S2 , RS- B/L AIR ENTRY



P/A-UTERUS W/C

L/E: NO ACTIVE BLEEDING, LOCHIA HEALTHY

B/L BREASTS: SOFT, NON TENDER

STOOL AND URINE PASSED.

ADVICE ON DISCHARGE:

FOR MOTHER:

- HIGH PROTEIN DIET
- TAB. CEFIXIME 200 MG BD FOR 3DAYS
- TAB. PANTOP 40MG OD ES FOR 3 DAYS
- TAB. PCM 500MG PO TDS X 3DAYS F/B SOS IF PAIN
- T. IRON FOLIC ACID OD FOR 3 MONTHS
- T. CALCIUM VITAMIN D BD FOR 6 MONTHS
- MAINTAIN PERINEAL HYGIENE
- PRACTICE KEGEL EXERCISES 2 SETS OF 25 EACH TWICE DAILY.
- SEXUAL ABSTINENCE FOR 6 WEEKS.
- HOME ISOLATION, WEAR MASK, SOCIAL DISTANCING, NO SOCIAL GATHERING, BE AT HOME.

FOR BABY

- 1) EXCLUSIVE BREASTFEEDING AND BURPING AFTER EACH FEED
- 2) IMMUNISATION AS PER SCHEDULE
- 3) CORD CARE/EYE CARE
- 4) FOLLOW UP AS PER ADVICE OF PEDIATRICIAN

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 19-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE : REVIEW IN GYNOC OPD AFTER 7 DAYS ON MONDAY/WEDNESDAY FOR SUTURE REMOVAL OR SOS IN CASE OF BLEEDING PV, SEVERE PAIN ABDOMEN, FOUL SMELLING DISCHARGE AND/OR FEVER, AND/OR BREAST PAIN.

REVIEW IN AYUSH COVID-OPD I/C/O COLD ,COUGH, FEVER, BREATHING DIFFICULTY , BODY PAINS AND/OR DIARRHEA.

Summary Prepared By

anonymous

Summary Approved By

anonymous